

Service Planning & Budgeting





Week 2

Service Plan Development Process



Learning Outcomes cont.

At the end of the session, the learner should be able to understand the

- Service Plan Development Process and its Key Components
- Step by Step Process of Service Planning in Health Services
- Setting Goals and Objectives
- Needs Assessment and Assessment of Existing Services
- Service Models and Service Profiles
- Budgeting & Financial Assessment in Service Planning
- Performance Assessment & Key Performance Indicators
- Implementation Monitoring and Evaluation





Service Plan Development Process: Overview

- The Main Component in Developing of a Service Plan





(01) Issue identification and goal setting

Goals and Objectives of Planning

The first step is to identify and confirm the issues to be addressed by the service planning exercise.

Few Examples:

- Insufficient capacity to respond to needs or demand
- Inadequate scope and range of services
- Poor access to the services provided
- Low quality and efficiency of service provision
- Inadequate structures, systems and technology required for service provision
- Need to improve operations in service delivery
- Need to improve the benefit or profitability of a service



Setting of Goals and Objectives in Service Planning

Goals: These are broad statements of intent that set the direction for the plan.

Objectives: These are the measurable outcomes of each goal expected to be achieved from the implementation of the service plan

Example

Goal:

The health service improves access for older patients

Objectives:

To have the capacity to admit persons over the age of 65

To admit persons over the age of 65 to the service within 30 minutes



Activity	Setting goals and objectives to the service plan
Rationale	To identify the desired future state(s) to result from undertaking the service planning exercise
Description	▪ Identify and categorise the desired future state(s), establish consensus within the planning team ▪ Translate the desired future state(s) into tangible goals and objectives
Outcome	Clear direction for planning process
Result	A Goal and Objectives Statement in the initial part of the Service Plan



(02) Contextual Analysis

Assess relevant government, corporate and organizational policies to ensure that planning accounts for regulations, guidelines and broader organizational, national or regional health directions.

Activity	Contextual Analysis
Rationale	To analyse, assess and reference relevant government, corporate and organisational policies, standards and guidelines for the services being investigated
Description	· Perform a comprehensive literature research to gather all relevant documents · Consult with the stakeholders and planning team for their policy views and strategic directions
Outcome	Regulations, guidelines, standards, policies and broader system goals are accounted for
Result	Output document of service planning process is in alignment with overall system health directions and related policies, guidelines and practice standards.



(03) Stakeholder Engagement

The success of planning is the relationship between health planners with stakeholders.

Activity	Stakeholder Engagement
Rationale	To gain the perspective and interests of a range of stakeholders within the plan's scope and assess their relative importance and influence
Description	▪ Identify stakeholders relevant to plan's scope ▪ Perform stakeholder analysis ▪ Consult and engage stakeholders using a variety of techniques
Outcome	Greater depth and breadth of understanding of issues affecting or impacting stakeholders consulted
Result	Issues Paper to outline issues arising from stakeholder groups consulted



(04) Catchment Identification

A catchment is defined as the population of a selected geographic area for which the services are to be provided. Catchment selection identifies the location of the service and the proximity of the population who are to use the services.

Activity	Catchment Identification
Rationale	To identify the location of the service and the proximity of the population who are to use the health service(s)
Description	▪ Define the location of the service(s) ▪ Define the primary, secondary and tertiary catchment areas for the service(s) ▪ Agree upon the catchment size and population ▪ Identify the population that the service(s) will serve in each catchment including main population of primary catchment and inflows ▪ Include a brief description of the geography, history and local economy of the area for each catchment ▪ Include a map if necessary
Outcome	Catchments are well defined and consensus within planning team is reached
Result	Planning process for service(s) is specific to defined catchments

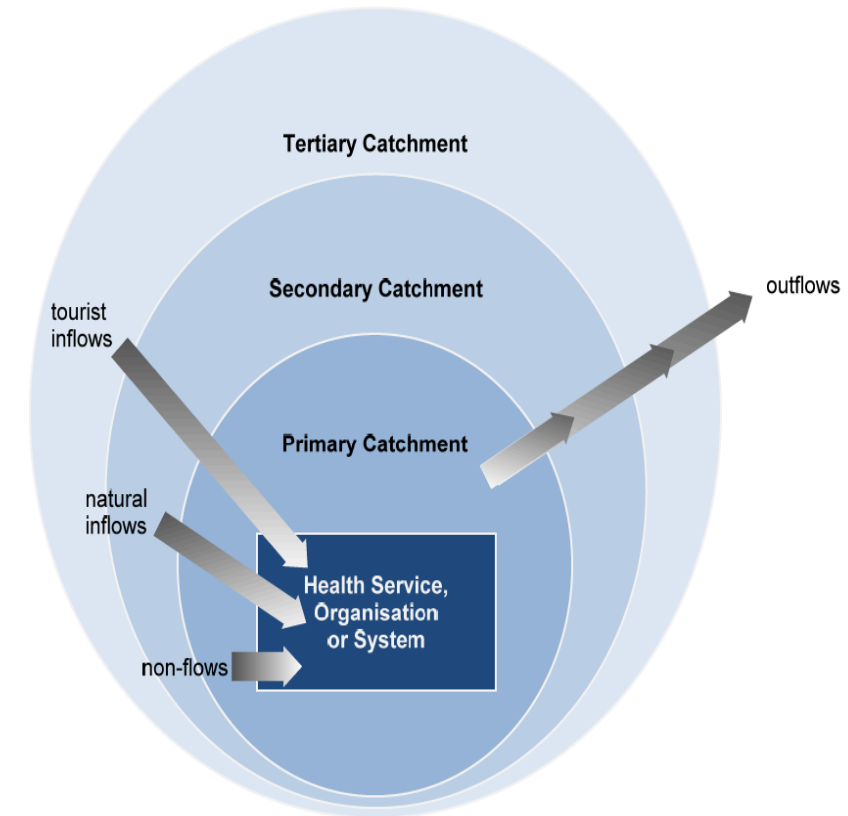


Figure 5 Catchment identification and types of patient movement

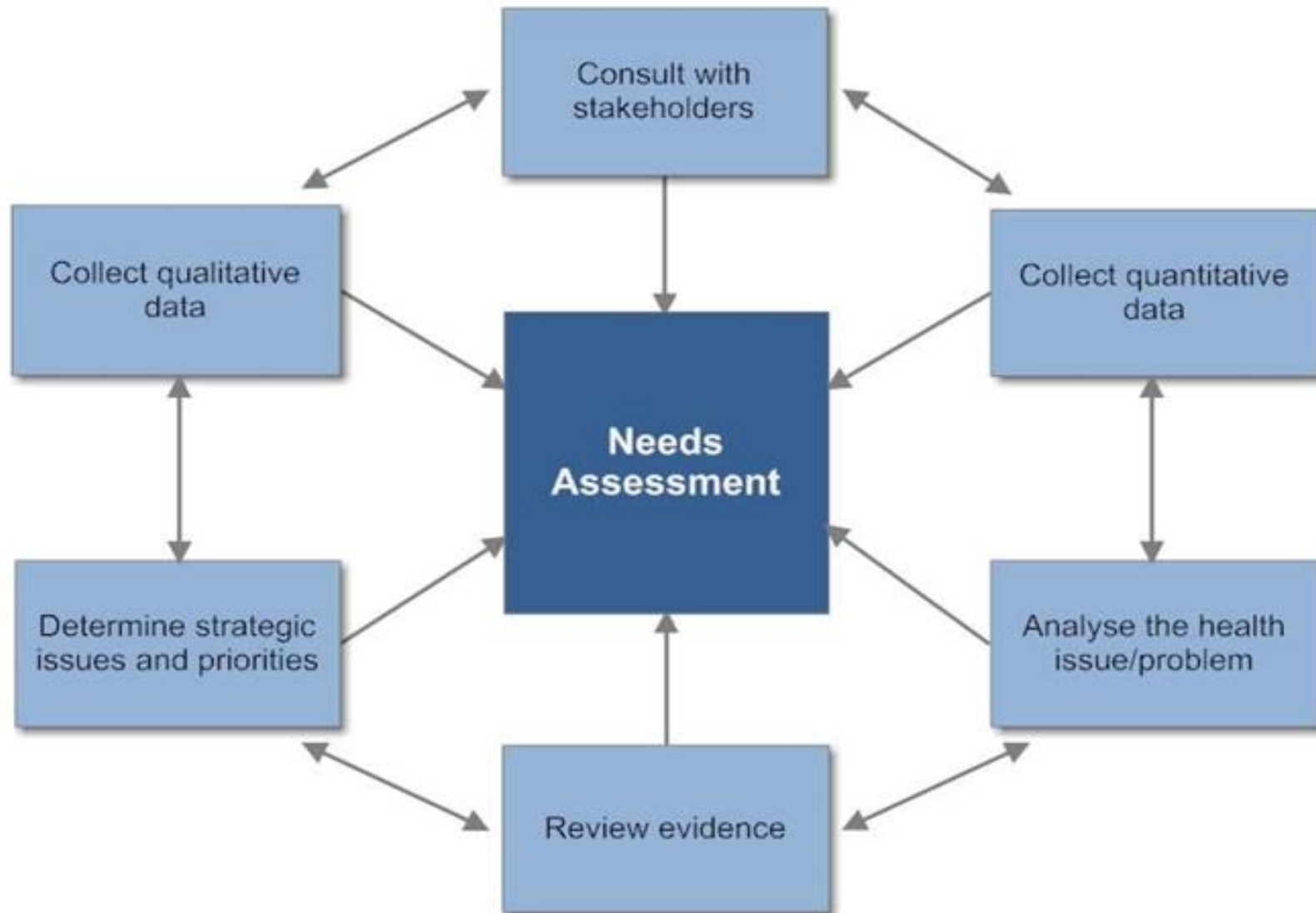


(05) Needs Assessment cont.

Activity	Needs Assessment
Rationale	To identify the health needs of the catchment population using quantitative and qualitative techniques and in consultation with experts in the selected health service(s)
Description	▪ Consultation with clinical experts, consumer representatives, researchers and service managers ▪ Describe demographic and socioeconomic characteristics of the population ▪ Assess region/nation-wide epidemiological and health status data ▪ Collect and analyse health service data ▪ Describe geographic factors impacting on need ▪ Supplement quantitative data with opinions and needs as expressed by health professionals, service providers and the greater community
Outcome	Comprehensive health needs profile is available for the selected catchment
Result	Needs statement

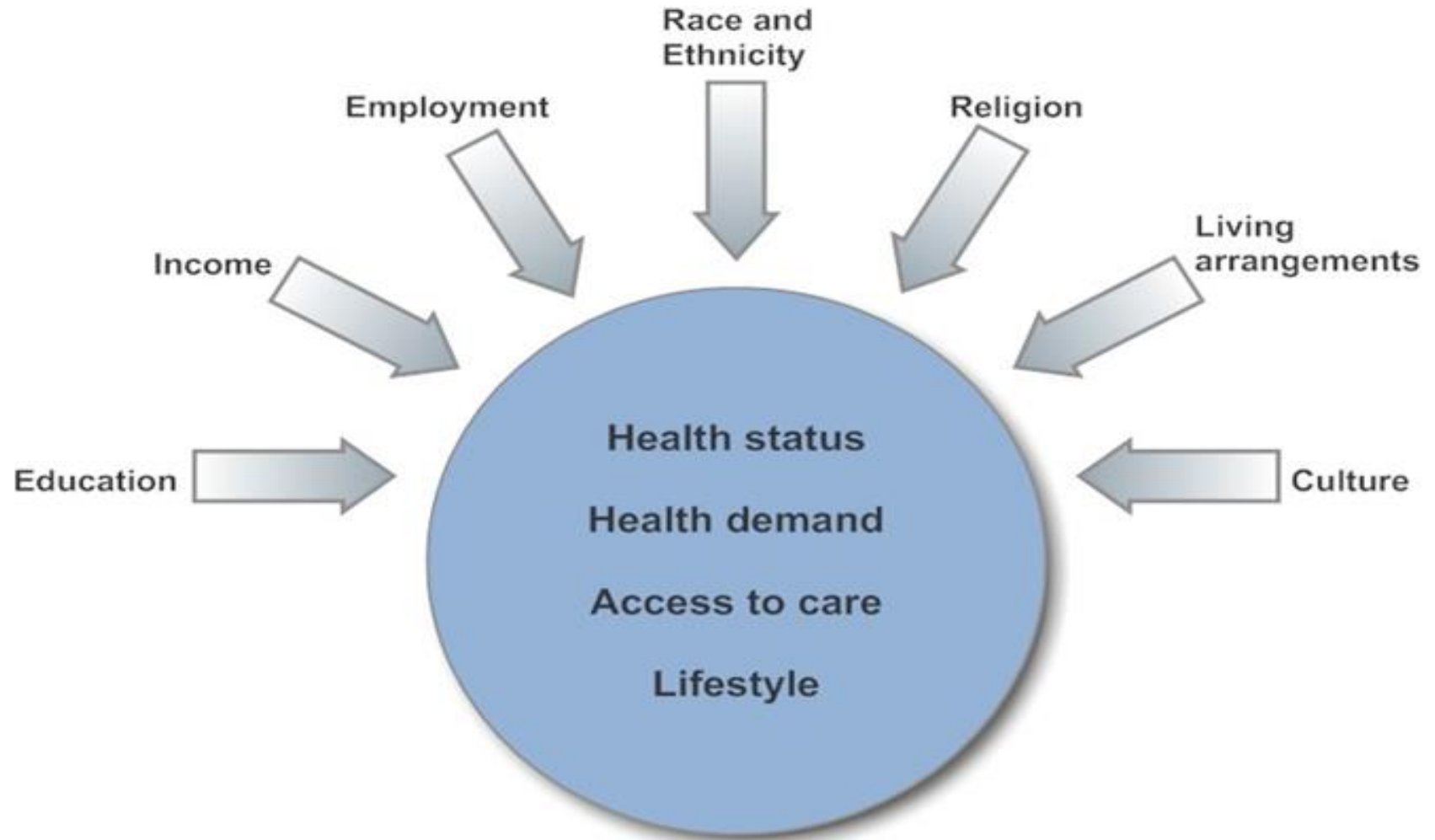


Needs Assessment cont.





Needs Assessment cont.





WHAT IS SWOT ANALYSIS?

SWOT is an acronym for Strengths, Weaknesses, Opportunities and Threats. Each letter is a vital component of the tool, which are laid out in a square format with four quadrants, called a SWOT matrix.

Project Name:

Surgery Department Improvement

Prepared By:

James Kennedy, C.I. Champion

Prepared Date:

2/11/2021

SWOT Analysis Matrix

Strengths	Weaknesses
• Offers specialized procedures that no other facilities in the state can • Physical layout of the unit is conducive for efficient patient care • Operational efficiency due to previous process excellence projects • High patient satisfaction	• Low satisfaction scores on employee surveys • High employee turnover • Dated equipment • Patient length of stay higher than average hospital stay • Operates at 75% capacity
Opportunities	Threats
• Market to other doctors who don't use the facility to generate more referrals • Increase patient awareness about the services offered • Certify all nurses as nurse anesthetists	• Competition from three other hospitals with similar procedures • Other hospitals exploring adding new procedures to compete better • Loss of revenue from out-of-network patients

Summary:

Strengths are highly unique compared to other hospitals in the region and are a differentiator when patients are looking for this procedure. Weaknesses demonstrate there is an opportunity to eliminate patient flow bottlenecks by having better trained doctors and staff. Despite current profits, revenue could be higher with strategic improvements.



(06) Existing Services

Existing services or supply of health services are analysed using several approaches, each giving differing results. These should then be jointly reviewed to gain a better understanding of the characteristics and number of existing services.

Activity	Identification of existing services
Rationale	To identify current health service supply, health service utilisation and service delivery models
Description	▪ Identify current health service supply ▪ Describe the services currently provided across the area ▪ Collect current health service utilisation data and analyse patterns and trends ▪ Describe the service models being used to deliver patient care ▪ Discuss health service delivery issues with key stakeholders and staff
Outcome	A clear understanding of current health service supply and utilisation and the models of care
Result	A profile of current health services



(07) Demand Projections

Some common methods for projecting future demand include **Trend Extrapolation, Benchmarking and Population Specific Computer Modelling**

Activity	Demand Projections
Rationale	To identify future demand for health services by service types and specialties
Description	▪ Define the base, target years and intervals for demand projections ▪ Obtain appropriate population projections for the catchment ▪ Consult and identify health service data for the defined catchment ▪ Use a projection technique to project the future health service demand in the projected population ▪ Produce forecasts based on projections adjusted and verified in expert consultation with service specialists
Outcome	A detailed statistical table of demand projections by service type, mode and specialty and agreed measures
Result	Health Service Demand Profile



(08) Gap Analysis

This is derived from the findings of the two components: **“Existing Services”** and **“Demand Projections”** to identify the gap between what health services are currently provided and what additional services need to be provided in the future.

Activity	Gap Analysis
Rationale	To identify the required changes to service capacity to address the health service need and demand to target year
Description	▪ Compare the findings from existing and proposed services and demand projections ▪ Identify the gap in health services and quantify as planning units including a mix of overnight beds, same day hospital beds, operating theatres, diagnostic units and ambulatory care units
Outcome	A detailed table of required health services and planning units to meet future health demands
Result	Service Gap Profile





(09) Service Model Selection

Following the identification and specifications of gaps in services, a number of service model options are generated to identify the **best option to address future needs**. Service models to be considered should be based on **best practice standards** and be **evidence-based**.

- **Benchmarks** – comparison of current models to models in other identical services that have achieved an excellent status
- **Health improvement** – in quality, effective service delivery and changes in practice, technology and therapies
- **Performance** – the preferred service model is a model that provides the best performance in cost, quality and time dimensions
- **Level of benefit achieved**



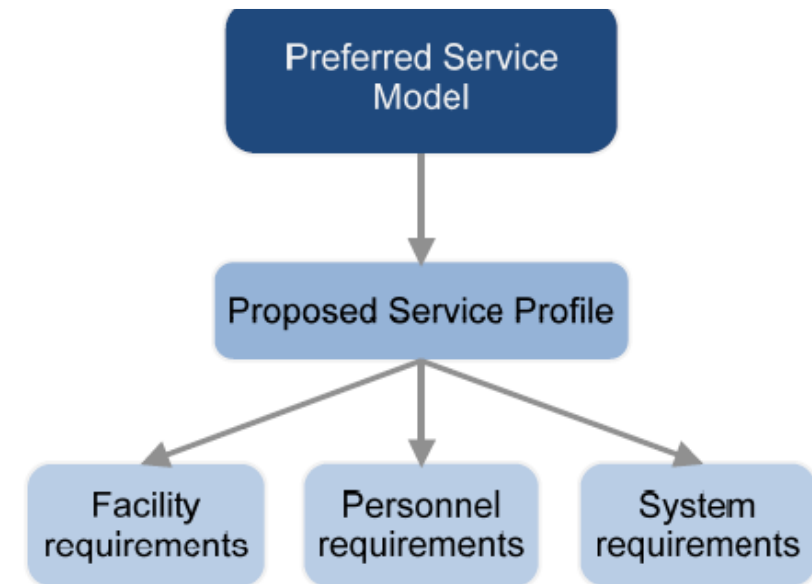
Activity	Service Model Selection
Rationale	To identify the service model to best address the health service gap identified
Description	▪ Develop service model options to address future demand for services ▪ Consider benchmarks, health improvement, performance and overall benefits achieved by each service model ▪ Consult with the stakeholders on the options and undertake formal selection procedures ▪ Select the preferred service model
Outcome	Preferred service model selected
Result	Preferred service model description



(10) Proposed Service Model Profiling

The selected service model is then translated into a service profile. This can be performed by quantifying the necessary physical, personnel and system resources required to deliver the service.

Activity	Proposed Service Model Profiling
Rationale	To translate the selected service model into service profile by quantifying all necessary requirements
Description	▪ Define specific resources needed in terms of physical, personnel and systems required ▪ Quantify the physical requirements as KPU's in forms of beds, chairs, rooms, cubicles, units, machines and devices by service type, mode and specialty
Outcome	Quantities of physical, personnel and system required
Result	Proposed service profile description





(11) Operations

The preferred service model and its service profile are used to describe functional requirements for the operation of the service.

Eg: Location, Hours of Operation, Legislation and Policy, Staff and Organisational Structure

Activity	Consider operational needs of the preferred service model and related service profile
Rationale	To identify and consider various operational components including: location, hours of operation, regulations, organisational structure and staffing, to operate services efficiently
Description	▪ Consult relevant stakeholders to identify location and hours of operation ▪ Define management structure and determine staff requirements ▪ Assess personnel and expertise supply and demand ▪ Determine appropriate action to match personnel demand and supply over course of plan
Outcome	Operational considerations of service profile taken into account
Result	Detailed documentation staff requirements according to hours of operation, personnel plan and management structure where relevant



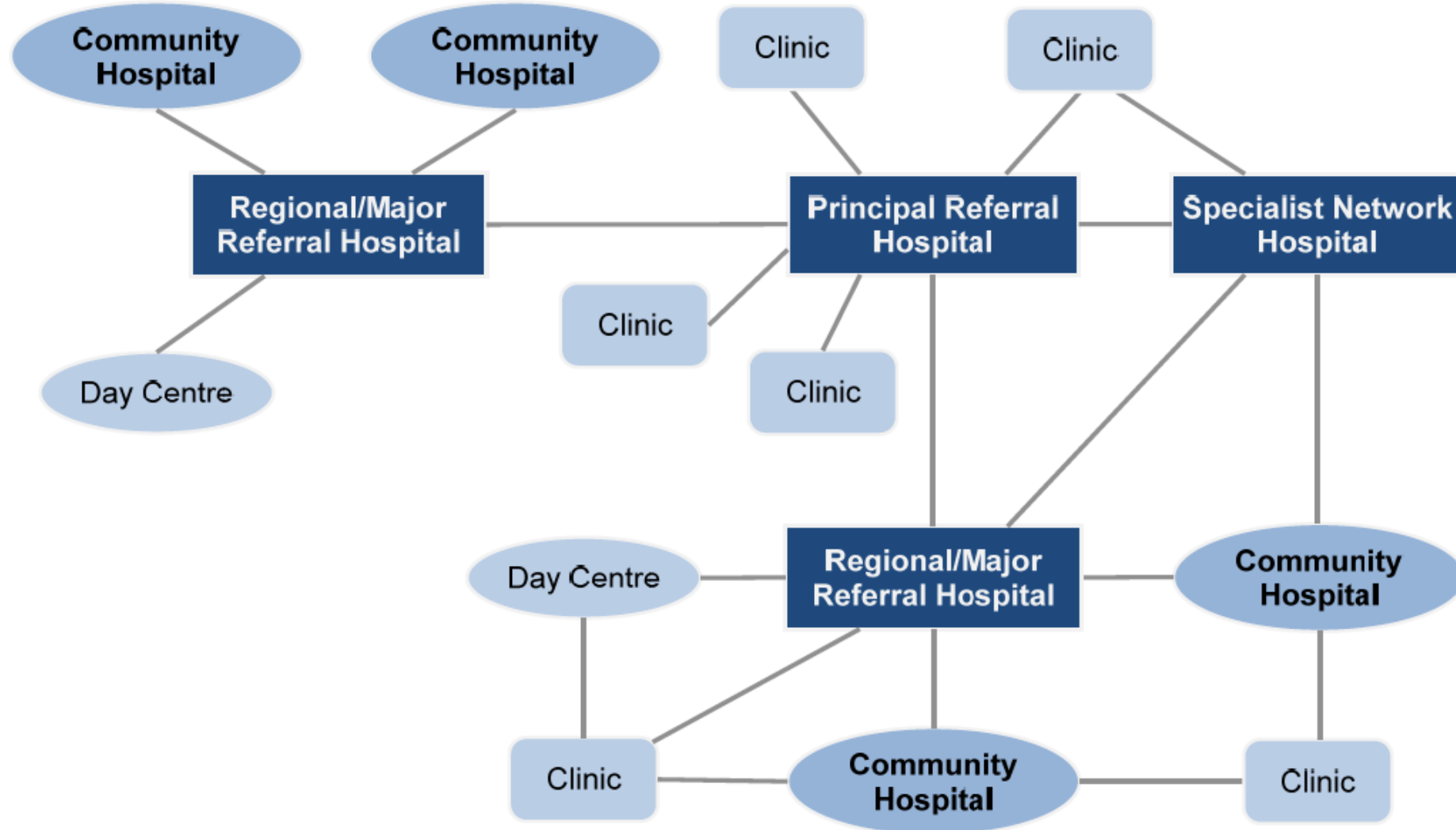
(12) Service Relationships

The delivery of the proposed service needs to be considered in the context of similar or related services. Planning should assess linkages to other health services both within the governing organization or region and within the general health system as a whole

Activity	Services and their relationships to one another
Rationale	To identify all service entities and demonstrate the relationships with one another
Description	▪ Identify the service entities ▪ Briefly profile their services and capacity if unknown ▪ Develop a relationship diagram to show the identified groups ▪ Describe the potential channels of patient referral and transfer between health services depending on the services provided
Outcome	Comprehensive knowledge of other health services within the area and where linkages may occur and a diagram showing relationships of services in a detailed view
Result	Documentation of potential channels of patient movement between health services and Relationship Diagram



Relationship Diagram for a Health Network





(13) Resource Allocation / Financial Assessment

One of the final steps in the planning process is budgeting; a vital step in the process

- Resource allocation is a major tool for planning, significantly affecting its capacity
- A rolling plan may be used to make realistic annual allocations as part of a longer-term plan of several years

BUDGET

- A capital and recurrent budget is to be determined for the preferred service model
- The budget should identify current services, proposed new services and the difference between the existing and proposed
- Savings or additional funds should be identified to determine the financial impact of the plan



The proposed budget should be clearly separated into recurrent and capital categories in the following format:

Recurrent	Staff Goods Services Maintenance Loan/Debts
Capital	Planning Services Construction Furnishings, Fixtures, Fittings and Equipment Major Equipment and Information Technology
Impact	Savings Additional funds

Table 4 Example of a simple budget



Activity	Financial Assessment
Rationale	To determine a capital and recurrent budget for the proposed services
Description	▪ Estimate capital and recurrent costs ▪ Determine savings or investments required ▪ Identify sources of funds where growth is required
Outcome	Comprehensive breakdown of expected resource allocations within the preferred service model
Result	Budget proposal to accompany preferred service model



Item	Capital	Recurrent
Buildings	Construction costs, land, purchase	Maintenance, small buildings, rent, rates, depreciation
Equipment and furnishings	Purchase of large new items	Maintenance replacement, hire, depreciation, small equipment items
Transport and travel	New vehicles	Maintenance, replacement, fuel, hire
Communications and information technology	Computers and internet, radio, telephone including installation	Maintenance, operating costs
Energy	Generator, solar panels, electricity connection to electric grid	Oil and other fuels
Water, sanitation, waste disposal	Installation, building costs	Maintenance
Food equipment	Kitchen	Food costs for staff/patient
Housekeeping	Equipment and buildings	Housekeeping supplies
Medical and laboratory supplies equipment	Theatre, diagnostic, treatment, buildings	Reagents and drugs for outpatients and inpatients, clinics
General administration	Office furniture, computers and other hardware	Stationery, record system, software and maintenance
Personnel	Initial training	Salaries and on-costs (pensions, statutory payments), refresher and ongoing training
Consultancy services	For project preparation	For specialists services
SOURCE OF FUNDING		
	Loans, subsidies, equity	Revenue, subsidies, savings

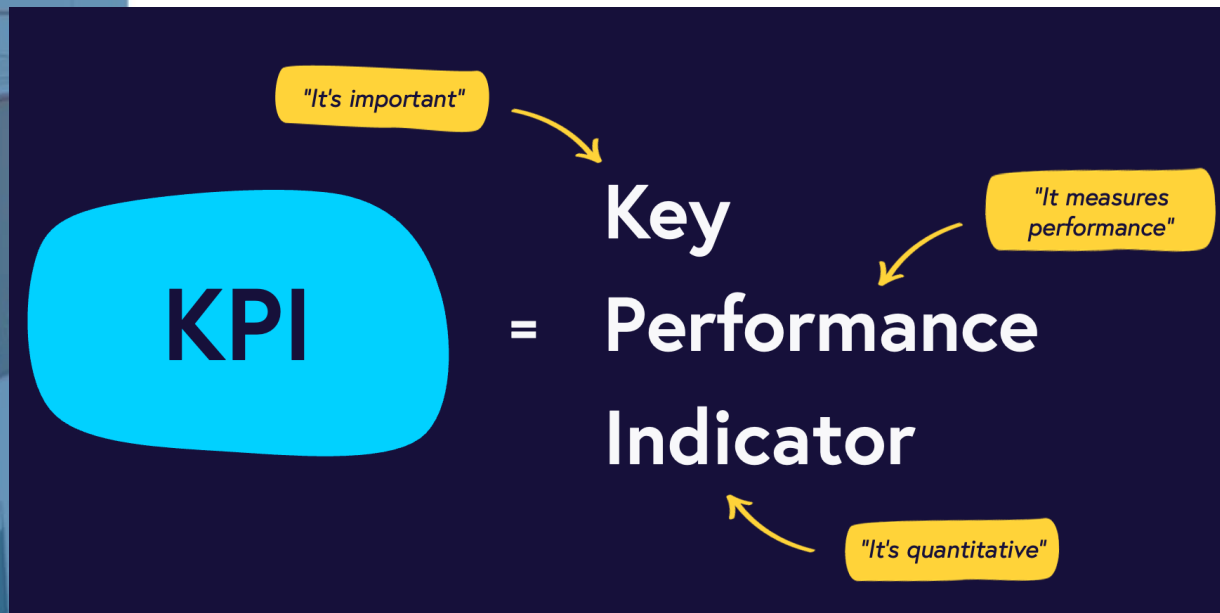
Table 5 Sample checklist for capital and recurrent budgets



(14) Performance Assessment

KEY Performance Indicators (KPI)

- To ensure the final service model addresses the goals and objectives of the planning process adequately, clear and measurable performance indicators should be formulated
- These indicators should clearly identify any constraints or resources required to achieve the agreed service model and align with those issued by higher governing authorities



- A quantifiable measure of performance over time for a specific objective.
- KPIs provide targets for teams to shoot for, milestones to gauge progress,
- Insights that help people across the organization make better decisions.

- Well-documented performance indicators and their associated targets, resource allocations, personnel responsible, and designated time frame, ensure that the plan will be more readily implemented and monitored

KPIs that always come out are:

Customer Satisfaction
Internal Process Quality
Employee Satisfaction
Financial Performance Index



Activity	Performance assessment
Rationale	To set measurable and attainable performance indicators with accompanying targets, resource allocations, responsible personnel and time frame
Description	▪ Identify performance indicator for each strategy of the Service Plan ▪ Negotiate realistic targets, resource allocations and time frames with personnel responsible and involved
Outcome	Documented performance indicators to measure and evaluate the success of the Service Plan
Result	Service Plan strategies are supplemented by performance indicators and targets



Getting approval for the service plan before implementing

The final draft Service Plan should be submitted to the governance team and supplied to contributing stakeholders and organizations, as well as senior management of facilities and health services for comment and approval prior to implementing final draft Service Plan should be submitted to the governance team and supplied to contributing stakeholders and organizations, as well as senior management of facilities and health services for comment and approval prior to implementation.



(15) Implementation

Implementation involves the translation of the service plan into a developmental or operational process. These functions are performed by service operators, service managers or an implementation team. A thorough handover of the plan facilitates the initial stages of implementation

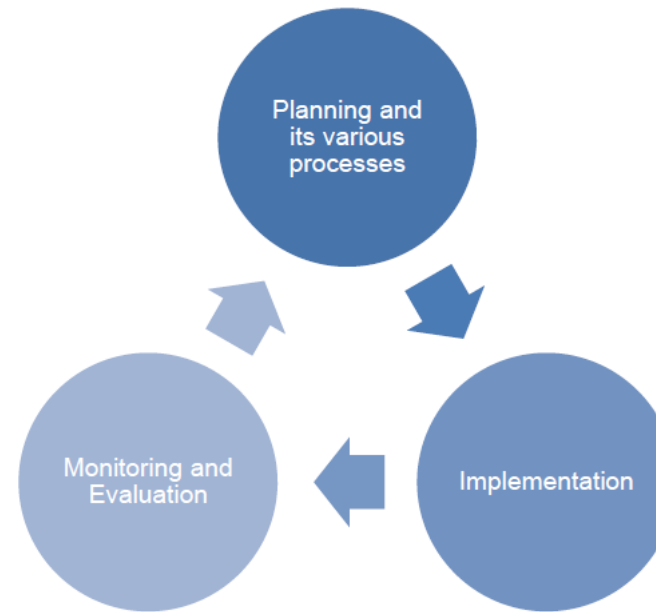
Activity	Service plan implementation
Rationale	To ensure the service plan and related documents are translated into developmental or operational processes
Description	▪ Identify implementation team ▪ Manage thorough handover from planning team to implementation team ▪ Ensure mechanisms for communication and negotiation exist to prevent implementation failure
Outcome	The goals and objectives of the service planning process are realised by appropriate change management and development processes
Result	Implementation plan supplementing service plan



(16) Monitoring and Evaluation

Monitoring during planning implementation allows the opportunity to adjust and review service plan strategies.

Activity	Monitoring and evaluation
Rationale	To assess the service planning and implementation process and to inform and assist future planning efforts
Description	▪ Assign evaluation team ▪ Undertake monitoring and evaluation techniques appropriate to service plan and implementation ▪ Report to planning and implementation teams the results of the assessments
Outcome	Assessment of planning and implementation processes and feedback for future planning efforts
Result	Evaluation report



The cyclical process of planning, implementation and monitoring and evaluation

Monitoring during planning implementation allows

- The opportunity to adjust and review service plan strategies
- To have progress reports that measure observations against the performance indicators
- To assist future planning efforts

Information - International Health Facility Guidelines © TAHPI Part S: Version 4.1: 2014



References and Further Reading

- AUSTRALASIAN HEALTH INFRASTRUCTURE ALLIANCE. 2012. *Australasian Health Facility Guidelines version 4* [Online]. Australia: Australasian Health Infrastructure Alliance. Available: <http://www.healthfacilityguidelines.com.au/default.aspx>.
- DEPARTMENT OF ENVIRONMENT AND PRIMARY INDUSTRIES. 2013. *Stakeholder Analysis (Stakeholder Matrix)* [Online]. Melbourne, VIC: State Government of Victoria. Available: <http://www.dse.vic.gov.au/effective-engagement/toolkit/tool-stakeholder-analysis-stakeholder-matrix> [Accessed 20 Jan 2013].
- EAGAR, K., GARRETT, P. & LIN, V. 2001. *Health planning: Australian perspectives*, Crows Nest, NSW, Allen & Unwin.
- GREEN, A. 2007. *An Introduction to Health Planning for Developing Health Systems*, New York, New York, Oxford University Press.
- KERZNER, H. 2013. *Project Management: A Systems Approach to Planning, Scheduling and Controlling* Hoboken, John Wiley & Sons.
- REICH, M. 1994. *Political Mapping of Health Policy: A Guide for Managing the Political Dimensions of Health Policy*, Boston, MA, Data for Decision Making Projects, Harvard School of Public Health.
- SPIEGEL, A. D. & HYMAN, H. H. 1998. *Strategic health planning: methods and techniques applied to marketing and management*, Norwood, New Jersey, Ablex Publishing Corporation.
- STANDARDS ASSOCIATION OF AUSTRALIA 1994. HB59: Ergonomics - the human factor, a practical approach to work systems design. Homebush, NSW: Standards Association of Australia.
- STATEWIDE SERVICES DEVELOPMENT BRANCH 2005. Area healthcare service plans - NSW health guide for development. Sydney, NSW: NSW Department of Health.
- THOMAS, R. 2003. *Health services planning*, New York, New York, Kluwer Academic/Plenum Publishers.
- VARVASOVSKY, Z. & BRUGHA, R. 2000. A stakeholder analysis. *Health Policy Plan*, 15, 338-345.



Student Activity

Based on the steps you learnt in “Service Planning”, let us do a group activity of Mapping a Service Plan for a selected health care service provider

Eg: New service or improvement in GP Surgery/ Care Home etc.